

DPP-4 Market Performance Tracker

Category Growth, Competitive Share, Growth Drivers and Volume-versus-Value Analysis in England

Executive Summary

This project develops a Power BI commercial performance tracker for the English DPP-4 prescribing market using publicly available NHS prescribing data. The analysis focuses on five major single-agent DPP-4 molecules: Sitagliptin, Linagliptin, Alogliptin, Saxagliptin and Vildagliptin.

The project was designed from a category-management perspective rather than a single-product perspective. The key objective is to understand how the DPP-4 market is evolving, which molecules are strengthening or weakening their competitive position, which molecules are driving net category change, and how prescription-volume share differs from recorded Net Ingredient Cost share.

The primary commercial comparison uses **January–July 2026 versus January–July 2025**, while historical trends cover **July 2024 to July 2026**.

The analysis finds that total DPP-4 prescription items were broadly stable, declining by **approximately 0.61% year-on-year** to around **3.96 million prescription items**. However, this stable category-level picture masks significant internal competitive movement.

Sitagliptin grew by **18.02% year-on-year**, reaching approximately **48.70% of DPP-4 prescription-item volume** and gaining **7.69 percentage points of share** relative to the prior-year period. By contrast, Linagliptin, Alogliptin, Saxagliptin and Vildagliptin all declined in prescription-item volume.

Sitagliptin added approximately **295,000 prescription items**, partially offsetting declines across the rest of the category. This demonstrates that net category growth alone does not explain the underlying competitive dynamics.

The project also identifies a substantial divergence between prescribing volume and recorded ingredient cost. DPP-4 recorded NIC declined by approximately **18.57% year-on-year**, considerably faster than prescription-item volume. Sitagliptin accounted for approximately **48.70% of category volume but only 7.04% of recorded NIC**, while Linagliptin represented a substantially larger share of recorded NIC than of prescription-item volume.

The final dashboard contains four analytical pages:

1. Market Overview
2. Competitive Performance
3. Category Growth Drivers
4. Volume vs Value

The project demonstrates pharmaceutical commercial analytics, competitive market analysis, Power Query transformation, DAX, time intelligence, data modelling, quality assurance, Figma-supported dashboard design and commercial interpretation.

1. Introduction

Commercial pharmaceutical analysis often requires understanding market performance at several levels simultaneously.

A category can appear stable in aggregate while individual molecules experience materially different growth trajectories. Similarly, a molecule can increase prescription volume while losing market share if competitors grow faster. Recorded ingredient cost may also move differently from prescribing volume.

This project applies these principles to the English DPP-4 prescribing market.

Unlike previous projects focused on geographic opportunity, disease burden or a single focal molecule, this analysis adopts a category-level competitive perspective.

The analytical hierarchy is:

Category → Molecule → Growth → Share → Contribution → Recorded Cost

The project focuses on a set of fundamental commercial measures:

- prescription items;
- absolute growth;
- year-on-year growth;
- market share;
- market share change;
- growth gap;
- contribution to net category change;
- Net Ingredient Cost;
- NIC per item;
- NIC share; and
- value-volume gap.

The objective is to produce a concise market-performance tracker that could support routine category review and competitive-performance discussions.

2. Business Context and Analytical Question

The project assumes the role of a commercial analyst supporting a diabetes business unit.

Management requires a clear understanding of the DPP-4 category rather than an opportunity-ranking or geographic segmentation exercise.

The primary business question is:

How has the English DPP-4 prescribing market evolved, and which molecules are driving changes in category volume, competitive share and recorded ingredient cost?

This is supported by several secondary questions:

- How large is the DPP-4 category?
- Is the total category growing or declining?
- Which molecule holds the largest prescription-item share?
- Which molecules are growing or declining fastest?
- Which molecules are gaining or losing competitive share?
- Which molecules contribute most to net category change?
- How does prescription-volume share differ from NIC share?
- How does recorded NIC per item differ across molecules?
- Does competitive importance change depending on whether volume or recorded cost is considered?

3. Data Source

The project uses publicly available data from the **NHS Business Services Authority English Prescribing Dataset**.

The source data records prescribing activity in England and includes fields such as:

- prescribing month;
- chemical substance code;
- chemical substance description;
- BNF presentation code;
- presentation description;
- prescription items;
- total quantity;
- Net Ingredient Cost;

- Actual Cost; and
- SNOMED code.

The raw source data is available at GP-practice level.

Because this project does not include geographic analysis, practice-level detail was not required in the final analytical model.

The data was therefore aggregated to a monthly presentation-level grain.

4. Analytical Scope

4.1 Time Period

The complete analytical dataset covers:

July 2024 – July 2026

This provides 25 monthly periods for historical analysis.

The headline performance comparison uses:

January–July 2026 versus January–July 2025

This ensures that equivalent seven-month periods are compared.

The dashboard therefore uses:

- Jan–Jul 2026 vs Jan–Jul 2025 for year-on-year KPIs;
- Jul 2024–Jul 2026 for monthly historical trends.

4.2 Market Definition

The primary market includes five single-agent DPP-4 molecules:

- Sitagliptin
- Linagliptin
- Alogliptin
- Saxagliptin
- Vildagliptin

Combination products were excluded from the primary analysis.

This was an intentional modelling decision.

DPP-4-containing combination products may include another therapeutic component such as metformin or an SGLT2 inhibitor. Combining these products with single-agent therapies would create a less consistent competitive denominator.

For this project:

Market share refers to prescription-item share within the defined five-molecule single-agent DPP-4 category.

Maintaining this definition consistently is essential because market size, market growth, market share and contribution metrics all depend on the denominator.

5. Data Preparation

The raw monthly prescribing files were prepared in Power Query.

The main workflow included:

1. importing the monthly prescribing datasets;
2. harmonising different source schemas;
3. standardising field names;
4. converting prescribing month fields into proper date values;
5. retaining relevant analytical fields;
6. correcting financial-field data types;
7. filtering to the defined five DPP-4 molecules;
8. excluding combination products;
9. appending monthly source files;
10. aggregating data to monthly presentation level;
11. creating dimension tables; and
12. validating data quality before building measures.

5.1 Source Schema Harmonisation

The raw monthly files did not all use identical schemas.

Different source groups were therefore standardised before append.

Fields were harmonised into the following final structure:

- YearMonth
- ChemicalCode
- Molecule
- PresentationCode
- PresentationName
- Items
- TotalQuantity
- NIC

- ActualCost
- SNOMEDCode

Standardising source structure before append reduced the risk of missing fields or inconsistent calculations across periods.

5.2 Date Transformation

Month values stored in YYYYMM format were converted into proper dates.

For example:

202407 → 01 July 2024

This enabled:

- chronological sorting;
- monthly trends;
- year-on-year comparisons; and
- Power BI time-intelligence functions.

A readable MonthYear field and a numeric YearMonthSort field were also created to maintain correct visual order.

5.3 NIC and Actual Cost Data Types

An important quality-assurance requirement was ensuring that NIC and Actual Cost were handled as decimal numeric fields.

These values were parsed using appropriate locale handling before append and aggregation.

This avoided the loss of decimal precision that can occur when financial values are incorrectly converted to whole numbers.

Fixed divisors were not applied.

Instead, the correction occurred at the data-type level so that the original decimal structure of the source data was preserved.

5.4 Aggregation

The final fact table was grouped using:

- YearMonth;
- ChemicalCode;
- Molecule;

- PresentationCode;
- PresentationName; and
- SNOMEDCode.

The following numeric fields were aggregated using sums:

- Items;
- TotalQuantity;
- NIC; and
- ActualCost.

The resulting analytical grain is:

one row per presentation per month

This provides sufficient detail for molecule and presentation analysis while avoiding unnecessary practice-level model size.

6. Data Model

The final Power BI model follows a simple star-schema structure.

The central fact table connects to three analytical dimensions:

Dim_Calendar → Fact_Prescribing ← Dim_Chemical

Dim_Presentation → Fact_Prescribing

6.1 Fact_Prescribing

The central fact table contains:

- YearMonth
- ChemicalCode
- Molecule
- PresentationCode
- PresentationName
- Items
- TotalQuantity
- NIC
- ActualCost
- SNOMEDCode

This table stores all additive commercial measures.

6.2 Dim_Calendar

The calendar dimension contains:

- YearMonth
- Year
- MonthNumber
- MonthName
- Quarter
- MonthYear
- YearMonthSort

This dimension supports chronological reporting and previous-year calculations.

6.3 Dim_Chemical

The chemical dimension contains:

- ChemicalCode
- Molecule
- TherapyClass
- DPP4Flag

The table contains one row for each of the five molecules included in the market.

6.4 Dim_Presentation

The presentation dimension contains:

- PresentationCode
- PresentationName
- ChemicalCode
- Molecule
- SNOMEDCode
- Formulation
- Strength
- PresentationGroup

Although the dashboard primarily focuses on molecule-level performance, maintaining a presentation dimension enables future drill-down analysis without rebuilding the data model.

7. Commercial KPI Framework

The project uses several complementary measures rather than relying on one headline metric.

7.1 Prescription Items

Prescription Items are used as the primary volume measure.

They represent prescribing activity.

They do not represent unique patients.

7.2 Absolute Growth

Absolute Growth is calculated as:

Current Prescription Items – Prior-Year Prescription Items

It answers:

How much prescribing volume was added or lost?

This measure is particularly important when comparing molecules of different sizes.

7.3 Year-on-Year Growth

YoY Growth is calculated as:

(Current – Prior Year) / Prior Year

It answers:

How fast is the molecule or category changing?

Absolute growth and percentage growth therefore answer different commercial questions.

7.4 Volume Share

Volume Share is calculated as:

Molecule Prescription Items / Total DPP-4 Prescription Items

It describes the molecule's current size within the defined competitive category.

7.5 Share Change

Share Change is:

Current Volume Share – Prior-Year Volume Share

It is expressed in percentage points.

This describes whether competitive position has strengthened or weakened.

7.6 Growth Gap

Growth Gap compares molecule performance with overall market performance:

Molecule YoY Growth – DPP-4 Market YoY Growth

A positive value indicates outperformance relative to the category.

A negative value indicates underperformance.

7.7 Contribution to Net Category Change

Contribution to Net Category Change is calculated as:

Molecule Absolute Growth / DPP-4 Absolute Growth

This measure identifies which molecules are driving or offsetting total category movement.

It can exceed 100% or become negative when positive and negative molecule-level movements offset each other.

7.8 Net Ingredient Cost

NIC is used as the project's primary recorded ingredient-cost measure.

NIC should not be interpreted as:

- manufacturer sales;
- manufacturer revenue;
- realised selling price;
- profit; or
- commercial margin.

7.9 NIC per Item

NIC per Item is calculated as:

Recorded NIC / Prescription Items

This provides a diagnostic measure of recorded cost intensity relative to prescribing activity.

It is not interpreted as an average manufacturer selling price.

7.10 NIC Share

NIC Share is calculated as:

Molecule NIC / Total DPP-4 NIC

This indicates how much of total category recorded ingredient cost is associated with each molecule.

7.11 Value-Volume Gap

The Value-Volume Gap is calculated as:

NIC Share – Volume Share

A positive gap indicates that the molecule accounts for a larger share of recorded NIC than of prescription-item volume.

A negative gap indicates the opposite.

The term “value” in this context refers specifically to recorded NIC and not to manufacturer commercial value.

8. Dashboard Design

The dashboard was designed using a combined Figma + Power BI workflow.

Figma was used primarily for:

- header design;
- visual identity;
- navigation structure;
- static background elements; and
- layout consistency.

Dynamic content remained native to Power BI.

This included:

- KPI cards;
- bar charts;
- line charts;
- dynamic molecule filtering;
- DAX-based calculations; and
- navigation.

The dashboard contains four navigation pages:

- Market Overview
- Competitive Performance
- Growth Drivers
- Volume vs Value

The design aims to resemble a concise commercial category-performance tracker rather than a highly exploratory analytical application.

9. Results

9.1 Page 1 — Market Overview

The first page answers:

What is happening to the DPP-4 market overall?

Headline metrics for Jan–Jul 2026 include:

- Total DPP-4 Items: approximately 3.96 million
- DPP-4 YoY Growth: -0.61%
- Total NIC: approximately £62.84 million
- NIC YoY Growth: -18.57%
- NIC per Item: approximately £15.86
- Leading Molecule: Sitagliptin

The page also includes:

- a monthly DPP-4 market trend;
- current molecule mix; and
- historical market-share trends by molecule.

Interpretation

At an aggregate level, the DPP-4 category remained broadly stable.

A **0.61% year-on-year decline** in prescription items represents only a relatively small change in overall prescribing volume.

However, the molecule-level mix changed materially.

Sitagliptin became the largest molecule in the category and represented approximately:

48.70% of total DPP-4 prescription items

This demonstrates that overall category stability does not imply stable competitive composition.

Recorded NIC also behaved differently from prescription volume.

NIC declined by approximately **18.57%**, substantially faster than the **0.61% decline in prescription items**.

This difference provided the rationale for the dashboard's later volume-versus-value analysis.

9.2 Page 2 — Competitive Performance

The second page answers:

Which molecules are gaining or losing competitive position?

The page includes a molecule selector.

When a molecule is selected, the following KPIs update dynamically:

- Selected Molecule Items
- Molecule YoY Growth
- DPP-4 Market Growth
- Growth Gap
- Volume Share
- Share Change

The competitive-market-share charts remain fixed to the entire DPP-4 category so that the selected molecule can still be interpreted within the wider competitive landscape.

Sitagliptin Example

For Jan–Jul 2026, Sitagliptin recorded:

- approximately 1.93 million prescription items;
- +18.02% YoY growth;
- 48.70% volume share;
- +7.69 percentage points of share gain; and
- a +18.63 percentage-point Growth Gap relative to the total DPP-4 market.

The overall market declined by approximately 0.61% during the same period.

Interpretation

Sitagliptin therefore substantially outperformed the category.

Its positive growth occurred while every other major molecule in the defined market recorded negative prescription-item growth.

The distinction between growth and share is important.

A molecule can grow in absolute terms yet lose share if its category expands more rapidly.

In Sitagliptin's case, growth was materially stronger than the category, meaning that both absolute volume and relative competitive position improved.

The share-change analysis confirms this result.

Sitagliptin gained approximately **7.69 percentage points**, while the other major molecules lost share.

9.3 Page 3 — Category Growth Drivers

The third page answers:

Which molecules are actually driving the category's net change?

Headline metrics include:

- DPP-4 Absolute Growth: approximately **-24,000 prescription items**
- DPP-4 Market YoY Growth: **-0.61%**
- Largest Positive Contributor: **Sitagliptin**
- Sitagliptin Absolute Growth: approximately **+295,000 prescription items**

The page compares:

- Absolute Growth by Molecule
- Contribution to Net Category Change
- Molecule YoY Growth

Absolute Growth

Sitagliptin added approximately:

+294,500 prescription items

while the other molecules declined.

Examples of negative absolute growth included:

- Linagliptin;
- Alogliptin;
- Saxagliptin; and
- Vildagliptin.

The combined declines were sufficient to offset Sitagliptin's positive growth and produce a small total category contraction.

Interpretation

This page demonstrates the difference between:

scale of change

and:

speed of change

YoY percentage growth measures the rate of change.

Absolute growth measures the actual amount of incremental or lost prescribing volume.

This distinction is important when comparing molecules of different starting sizes.

Contribution to Net Category Change

Contribution measures appear unusually large in this period because total DPP-4 net change is relatively small compared with the individual molecule movements.

Sitagliptin's positive absolute growth substantially exceeded the total category's net decline.

As a result, its contribution-to-net-category-change ratio becomes a large negative percentage when divided by the negative category denominator.

This does not mean Sitagliptin contributed negatively to performance.

Instead:

Sitagliptin added substantial prescribing volume and partially offset declines recorded across the remaining DPP-4 molecules.

Similarly, molecules with negative absolute growth produce positive contribution percentages to the total category decline.

This illustrates why contribution measures require interpretation alongside their numerator and denominator rather than being read in isolation.

9.4 Page 4 — Volume vs Value

The fourth page asks:

Does competitive importance change when recorded ingredient cost is considered instead of prescription volume?

The page includes four selected-molecule KPIs:

- Prescription-Item Share
- Recorded NIC Share

- Value-Volume Gap
- Recorded NIC per Item

It also includes:

- Volume Share vs NIC Share by Molecule
- NIC per Item by Molecule
- Value-Volume Gap by Molecule

Sitagliptin Example

During Jan–Jul 2026, Sitagliptin represented approximately:

48.70% of DPP-4 prescription items

but only:

7.04% of recorded NIC

This creates a Value-Volume Gap of approximately:

-41.66 percentage points

Sitagliptin's recorded NIC per item was approximately:

£2.29

This was substantially lower than several other molecules.

Linagliptin Example

Linagliptin showed the opposite pattern.

Its share of recorded NIC was materially higher than its prescription-item share.

This produced a large positive Value-Volume Gap.

The relationship illustrates how a molecule's importance can appear different depending on whether the category is viewed through:

- prescription activity; or
- recorded ingredient cost.

Interpretation

The volume-versus-value analysis does not provide information about profitability.

A larger NIC share should not be interpreted as:

- higher revenue;
- higher margin;
- stronger profitability; or
- higher commercial price.

The analysis instead demonstrates that average recorded NIC per prescription item differs materially across molecules.

These differences contribute to the divergence between volume share and NIC share.

10. Overall Commercial Interpretation

Taken together, the four dashboard pages provide a consistent commercial story.

First, the DPP-4 market was broadly stable in prescription volume.

A **0.61% decline** suggests limited overall category movement.

However, this aggregate view conceals substantial internal competitive change.

Second, Sitagliptin materially strengthened its competitive position.

It recorded:

- positive volume growth;
- the largest absolute increase in prescription items;
- substantial share gain; and
- strong growth relative to the category.

Third, declines in the other major DPP-4 molecules offset Sitagliptin's growth.

The category's small net decline was therefore the result of much larger positive and negative molecule-level movements cancelling each other out.

Fourth, recorded NIC changed substantially faster than prescribing volume.

This means that category performance looks different depending on whether analysis focuses on:

- prescription-item activity; or
- recorded ingredient cost.

Finally, Sitagliptin's large volume share but comparatively small NIC share demonstrates why prescription volume and recorded cost should not be interpreted interchangeably.

The overall analytical pattern is therefore:

Stable category volume → major competitive mix shift → Sitagliptin share gain → offsetting declines elsewhere → materially different NIC dynamics

11. Critical Evaluation

The project provides a commercially useful market-performance framework, but several methodological considerations are important.

11.1 Market Definition

The market excludes DPP-4-containing combination therapies.

This improves denominator consistency but means the dashboard should not be interpreted as representing all medicines containing DPP-4 inhibitors.

The results specifically describe:

the defined single-agent DPP-4 market

Changing this definition would change:

- market size;
- market growth;
- molecule share;
- share change; and
- contribution calculations.

11.2 Items as a Volume Measure

Prescription Items provide a convenient common prescribing-activity measure.

However, different molecules may have different prescribing and dosing patterns.

Therefore item-based share is not necessarily equivalent to:

- patient share;
- treatment-equivalent share; or
- therapeutic-unit share.

11.3 Contribution Metrics

Contribution to net category change becomes difficult to interpret when the denominator is small.

In this project, the total category changed relatively little while individual molecule movements were substantially larger.

This produces very large positive and negative contribution percentages.

The measure remains mathematically valid, but it requires clear contextual explanation.

11.4 Cost Interpretation

NIC is a recorded ingredient-cost measure.

It does not provide manufacturer commercial outcomes.

Consequently:

NIC share should not be treated as revenue share.

Similarly:

NIC per Item should not be treated as average selling price.

12. Limitations

The project has several important limitations.

Prescription Items Are Not Patients

The analysis measures prescribing activity rather than unique patients.

One patient may account for multiple prescription items.

NIC Is Not Revenue

Recorded NIC does not represent:

- manufacturer sales;
- revenue;
- profitability;
- margin;
- rebates;
- discounts; or
- realised selling price.

Item-Based Shares Are Not Therapeutic-Equivalent Shares

Different medicines may have different prescribing patterns.

Therefore, one prescription item for one molecule should not automatically be considered therapeutically equivalent to one prescription item for another.

Combination Products Are Excluded

The primary analytical market contains only five single-agent DPP-4 molecules.

Combination therapies are outside the defined market.

England Scope

The source data represents English NHS prescribing activity.

It should not be interpreted as total UK pharmaceutical use.

No Patient-Level Analysis

The dataset does not provide longitudinal patient-level data.

The analysis cannot identify:

- patient switching;
- treatment initiation;
- adherence;
- persistence; or
- treatment sequence.

No Causal Attribution

The project identifies patterns in:

- growth;
- market share;
- share change;
- recorded NIC;
- NIC per item; and
- molecule mix.

It does not establish formal causal relationships between these measures.

13. Dashboard and User Experience Design

The dashboard uses a consistent four-page navigation system:

Market Overview | Competitive Performance | Growth Drivers | Volume vs Value

The pages are designed around distinct analytical questions.

Market Overview

Provides overall category size, growth and molecule mix.

Competitive Performance

Provides molecule-level relative performance using a dynamic molecule selector.

Growth Drivers

Explains which molecules are adding or losing prescribing volume.

Volume vs Value

Compares prescription-volume importance with recorded-cost importance.

Figma was used primarily for static header and navigation design, while analytical visuals remained native to Power BI.

This approach preserved flexibility while maintaining consistent visual identity.

14. Skills Demonstrated

The project demonstrates practical application of:

- pharmaceutical commercial analytics;
- category-performance analysis;
- competitive market analysis;
- market-share analysis;
- growth-gap analysis;
- absolute-growth analysis;
- contribution analysis;
- volume-versus-value analysis;
- recorded-cost analysis;
- Power BI;
- DAX;
- Power Query;
- time intelligence;
- star-schema modelling;
- data-quality assurance;
- Figma dashboard design; and
- commercial analytical storytelling.

The project also reinforces an important analytical principle:

A market can appear stable in aggregate while undergoing major internal competitive change.

15. Conclusion

This project developed a Power BI commercial-performance tracker for the English single-agent DPP-4 prescribing market.

The analysis identified a category that was broadly stable in aggregate but undergoing substantial competitive restructuring.

Total DPP-4 prescription items declined by approximately **0.61% year-on-year**, yet Sitagliptin grew by **18.02%**, reached approximately **48.70% volume share**, and gained **7.69 percentage points of competitive share**.

Sitagliptin also contributed approximately **295,000 additional prescription items**, partially offsetting declines in the remaining major molecules.

The analysis therefore demonstrates why category-level growth alone is insufficient for understanding competitive market performance.

Recorded ingredient cost added a second analytical perspective.

DPP-4 NIC declined by approximately **18.57%**, considerably faster than prescription volume.

Sitagliptin accounted for almost half of category prescription-item volume but only around **7% of recorded NIC**, highlighting a large divergence between prescribing-volume share and recorded-cost share.

The project shows how a relatively simple set of commercial measures can produce a comprehensive category-performance story when interpreted together:

- market size;
- growth;
- absolute change;
- competitive share;
- share change;
- contribution;
- NIC share; and
- NIC per item.

More broadly, the project demonstrates how publicly available NHS prescribing data can be transformed into a structured pharmaceutical commercial-analysis tool using Power BI, rigorous data preparation and disciplined interpretation.

Appendix A — Selected DAX Measures

Total Items

Total Items =

SUM(Fact_Prescribing[Items])

NIC

NIC =

SUM(Fact_Prescribing[NIC])

NIC per Item

NIC per Item =

DIVIDE(

[NIC],

[Total Items]

)

DPP-4 Market Items

DPP4 Market Items =

CALCULATE(

[Total Items],

REMOVEFILTERS(Dim_Chemical)

)

Volume Share

Volume Share =

DIVIDE(

[Total Items],

[DPP4 Market Items]

)

Volume Share Change

Volume Share Change (pp) =

VAR PreviousShare =

[PY Volume Share]

RETURN

IF(

NOT ISBLANK(PreviousShare),

([Volume Share] - PreviousShare) * 100

)

Growth Gap

Growth Gap (pp) =

VAR MoleculeGrowth =

[Items YoY Growth]

VAR MarketGrowth =

[DPP4 Market YoY Growth]

RETURN

IF(

NOT ISBLANK(MoleculeGrowth)

&& NOT ISBLANK(MarketGrowth),

(MoleculeGrowth - MarketGrowth) * 100

)

Contribution to Net Category Change

Contribution to Net Category Change =

VAR MoleculeGrowth =

[Absolute Items Growth]

VAR MarketGrowth =

[DPP4 Absolute Growth]

RETURN

IF(

```

NOT ISBLANK(MoleculeGrowth)

    && NOT ISBLANK(MarketGrowth),

DIVIDE(

    MoleculeGrowth,

    MarketGrowth

)

)

```

NIC Share

NIC Share =

```

DIVIDE(

    [NIC],

    [DPP4 Market NIC]

)

```

Value-Volume Gap

Value-Volume Gap (pp) =

$([NIC\ Share] - [Volume\ Share]) * 100$

Appendix B — Data Model & Dashboard

Figure 1. Power BI Data Model

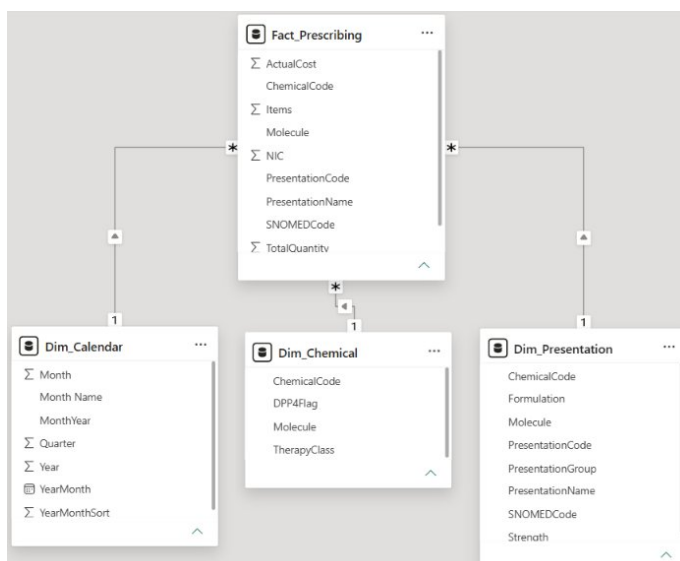


Figure 2. DPP-4 Market Overview Dashboard

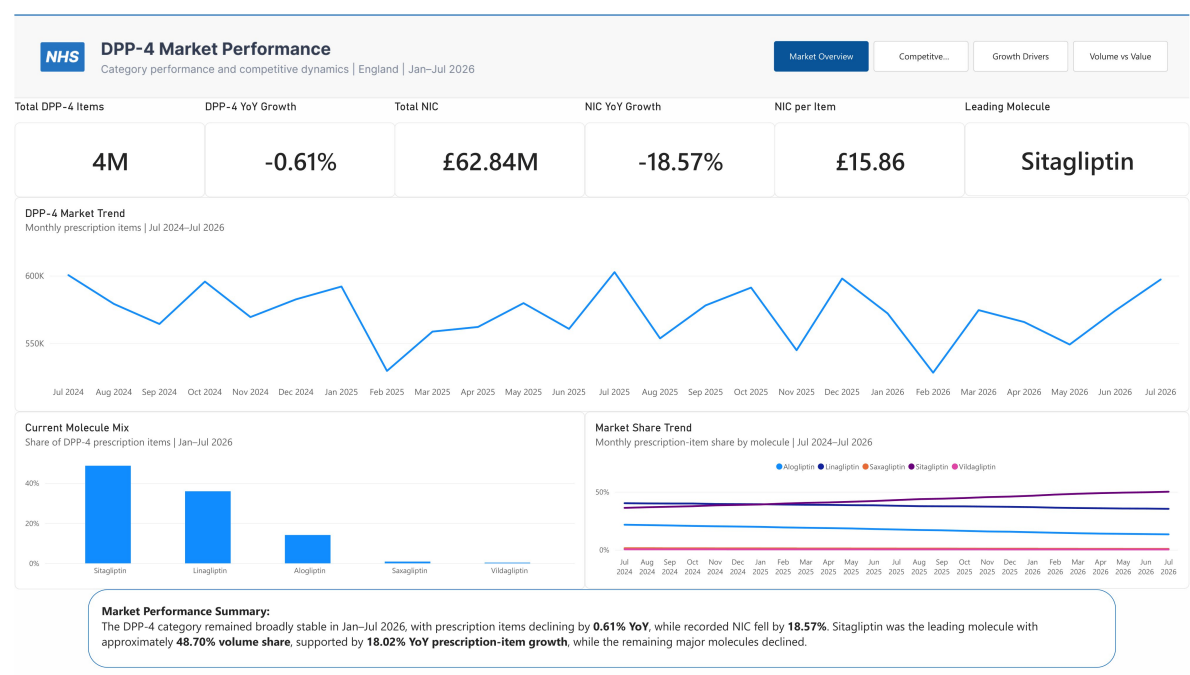


Figure 3. Competitive Performance Dashboard

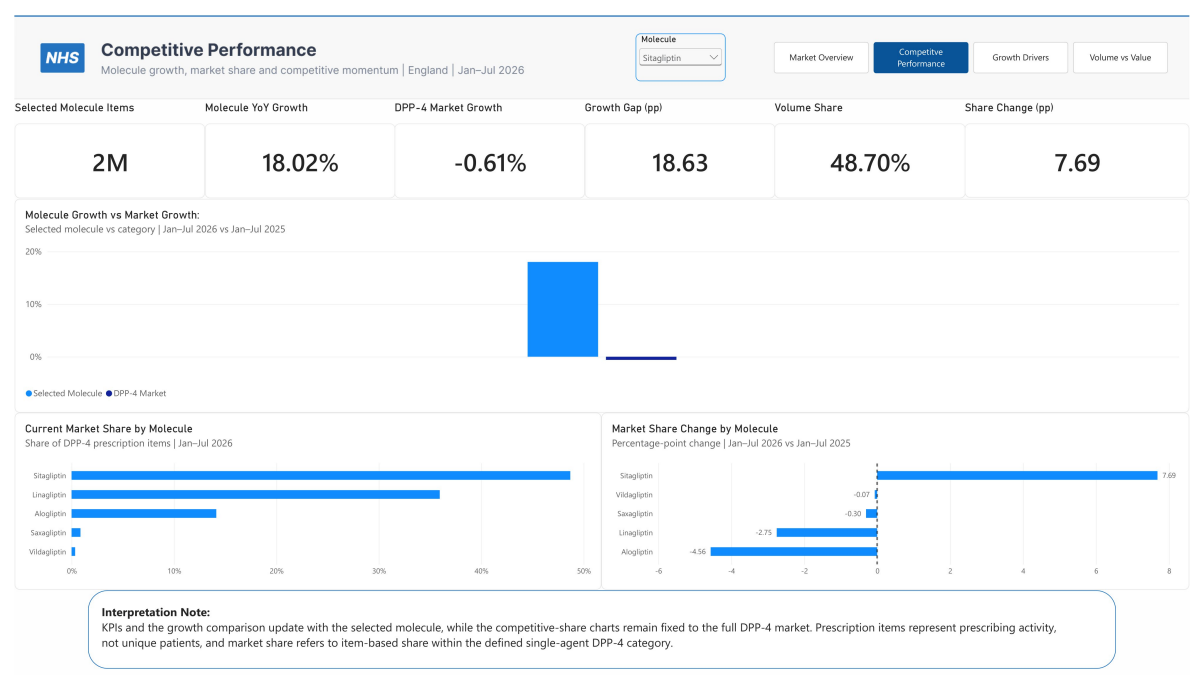


Figure 4. Category Growth Drivers Dashboard

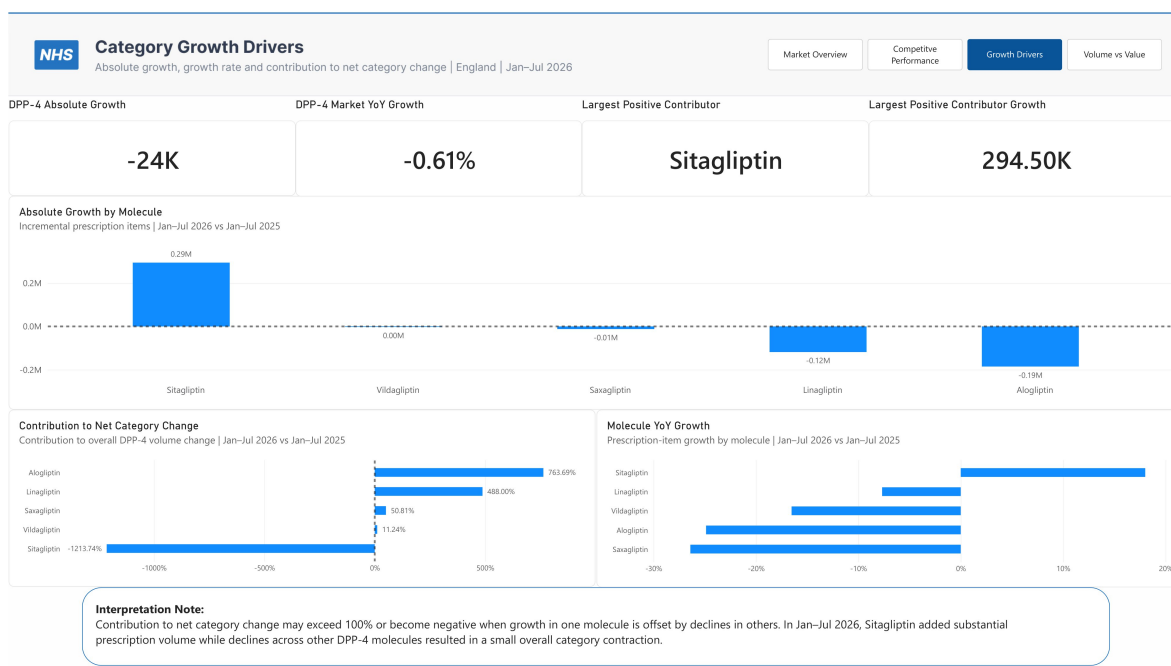
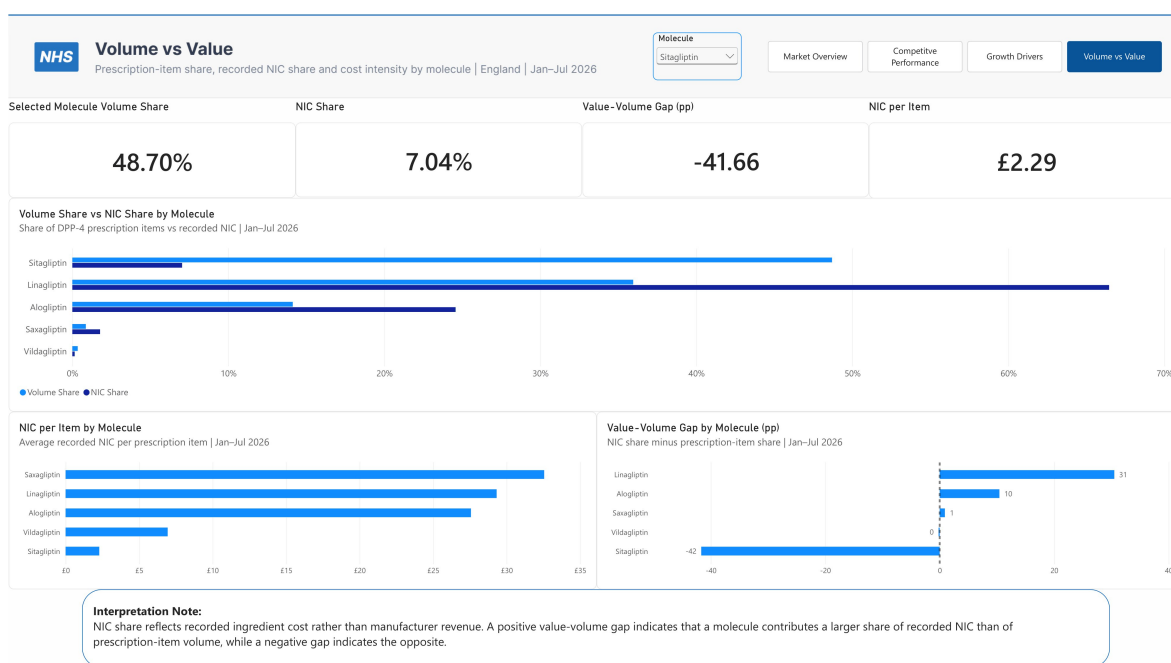


Figure 5. Volume vs Value Dashboard



Data Source

NHS Business Services Authority. **English Prescribing Dataset.**

Publicly available NHS prescribing data were used for independent educational and portfolio analysis.

Disclaimer

This project was developed independently for educational and portfolio purposes.

It is not affiliated with, endorsed by, or produced on behalf of the NHS, any pharmaceutical manufacturer, or any other commercial organisation.

The analysis is intended to demonstrate pharmaceutical commercial analytics, data modelling, Power BI, DAX, data-quality assurance and dashboard-design capabilities.

It should not be interpreted as clinical, financial, investment or commercial advice.